HCP Perception Survey: Trodelvia in triple-negative breast cancer (US, 2022)

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Executive Summary

This survey of 150 oncologists across United States examined how clinicians perceive Trodelvia (sacitodrinan) for triple-negative breast cancer. Fieldwork took place in January 2022.

Overall HCP sentiment index for Trodelvia in United States: 0.28 (scale 0 to 1, assessed 2022). Respondents viewed the brand negatively.

Methodology

Respondents were oncologists who treat at least 22 patients per month with triple-negative breast cancer. The sample size was n=150. Interviews lasted 23 minutes and were conducted online.

Results are weighted to national prescribing volumes. Statistical significance was assessed at the 95% confidence level.

Sequencing after progression remains an open question; oncologists in United States reported using a different mechanism of action after Trodelvia fails.

Biosimilar and generic competition in adjacent classes is shaping price expectations, even where Trodelvia has no direct substitute.

The unmet need in triple-negative breast cancer persists in patients who relapse or become refractory, where options are limited and outcomes remain poor.

Respondents expect real-world persistence on Trodelvia to be lower than in trials, mainly because of discontinuation after early adverse events and the complexity of managing fatigue.

Cost-effectiveness submissions typically model second-line use, and payers question the durability of benefit beyond the trial follow-up period.

Payer decisions increasingly rely on health technology assessment, requiring evidence of clinical benefit versus the standard of care and cost-effectiveness analyses.

Peer-to-peer education and congress data were cited as the most trusted sources when oncologists evaluate new evidence on TROP2 antibody-drug conjugate agents.

Medical education priorities include recognising infusion reactions early, communicating realistic expectations, and coordinating with primary care.

Key Findings

Oncologists in United States rated Trodelvia tolerability profile as negatively, citing immune-related adverse events and pneumonitis as the main concerns among patients with triple-negative breast cancer.

Adherence concerns centred on monitoring requirements, particularly in patients with comorbidities and polypharmacy.

Respondents highlighted the mechanism of action of Trodelvia (delivers a cytotoxic payload to trop2-expressing cells) as a differentiator in triple-negative breast cancer, although payer

prior-authorisation requirements slow initiation.

Many oncologists said they would like more information about Trodelvia.

The competitive set is evolving as new mechanisms enter phase 3 development, with readouts expected over the next two to three years.

Guideline committees are expected to review the evidence base for triple-negative breast cancer again next year, which could change eligibility criteria and referral patterns.

The unmet need in triple-negative breast cancer persists in patients who relapse or become refractory, where options are limited and outcomes remain poor.

Education needs include biomarker testing, management of adverse events and counselling patients on the expected benefit-risk.

Regional variation within United States is notable: large centres report faster uptake and broader use than smaller regional hospitals.

Sequencing after progression remains an open question; oncologists in United States reported using a different mechanism of action after Trodelvia fails.

Respondents expect real-world persistence on Trodelvia to be lower than in trials, mainly because of injection-site discomfort and the complexity of managing pneumonitis.

Access to specialist centres influences adoption: academic centres adopt new therapies faster than community practices, where formulary restrictions and reimbursement criteria apply.

Conclusions

Trodelvia is perceived negatively in United States. Priorities for the brand team are monitoring requirements and evidence generation for triple-negative breast cancer.

The competitive set is evolving as new mechanisms enter phase 3 development, with readouts expected over the next two to three years.

Cost-effectiveness submissions typically model refractory disease use, and payers question the durability of benefit beyond the trial follow-up period.

Safety monitoring focused on pneumonitis and fatigue, with dose interruption or discontinuation used for severe adverse events.

Access to specialist centres influences adoption: academic centres adopt new therapies faster than community practices, where formulary restrictions and reimbursement criteria apply.

Combination therapy and sequencing strategies remain debated; respondents noted limited head-to-head evidence between Trodelvia and competing agents.

Supply and logistics are not a concern for Trodelvia, though cold-chain handling and nurse-led administration add workload in busy clinics.

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Patient advocacy groups are pushing for earlier access, and shared decision-making tools are being piloted to explain benefit, risk and treatment burden.

Synthetic document generated for the Watson prototype. All brand names and figures are fictional.